

FRONT

DOMPERIDONE TABLET 10 mg

VISMO01-M

Description

Round, white to off-white coated tablet with shallow convex faces and bevel-edged.

Composition

Each tablet contains Domperidone 10 mg

Actions and Pharmacology

Domperidone is a dopamine antagonist with anti-emetic properties.

Domperidone does not readily cross the blood-brain barrier. In domperidone users, especially adults, extrapyramidal side effects are very rare, but domperidone promotes the release of prolactin from the pituitary. Its anti-emetic effect may be due to a combination of peripheral (gastrokinetic) effects and antagonism of dopamine receptors in the chemoreceptor trigger zone, which lies outside the blood-brain barrier in the area postrema.

Studies in man have shown oral domperidone to increase lower oesophageal pressure, improve antroduodenal motility and accelerate gastric emptying.

There is no effect on gastric secretion.

Pharmacokinetics

Absorption

In fasting subjects, domperidone is rapidly absorbed after oral administration, with peak plasma concentrations at 30 to 60 minutes. The low absolute bioavailability of oral domperidone (approximately 15%) is due to an extensive first-pass metabolism in the gut wall and liver. Although domperidone's bioavailability is enhanced in normal subjects when taken after a meal, patients with gastro-intestinal complaints should take domperidone 15-30 minutes before a meal. Reduced gastric acidity impairs the absorption of domperidone. Oral bioavailability is decreased by prior concomitant administration of cimetidine and sodium bicarbonate. The time of peak absorption is slightly delayed and the AUC somewhat increased when domperidone is taken after a meal.

Distribution

Small amounts of domperidone are distributed into breast milk, reaching concentrations about one-quarter of those in maternal serum.

Oral domperidone does not appear to accumulate or to induce its own metabolism. Domperidone is 91- 93% bound to plasma proteins. It does not readily cross the blood-brain barrier.

Metabolism

Domperidone undergoes rapid and extensive hepatic metabolism by hydroxylation and N-dealkylation. *In vitro* metabolism experiments with diagnostics inhibitors revealed that CYP3A4 is a major form of cytochrome P-450 involved in the N-dealkylation of domperidone, whereas CYP3A4, CYP1A2 and CYP2E1 are involved in domperidone aromatic hydroxylation.

Elimination

Domperidone is more than 90% bound to plasma proteins, and has a terminal elimination half-life of about 7.5 hours. It is chiefly cleared from the blood by extensive metabolism. About 30% of an oral dose is excreted in urine within 24 hours, almost entirely as metabolites; the remainder of a dose is excreted in faeces over several days, about 10% as unchanged drug.

Indications

For the relief of post-prandial symptoms of fullness, nausea, epigastric bloating and belching that is occasionally accompanied by epigastric discomfort and heartburn.

Contraindications

Domperidone Tablet is contraindicated for patients of:

- Known hypersensitivity to domperidone or any of the excipients.
- Prolactin-releasing pituitary tumour (prolactinoma).
- Gastro-intestinal haemorrhage, mechanical obstruction, perforation or when stimulation of the gastric motility could be harmful.
- Hepatic and/or renal impairment.

Precautions

Domperidone is not recommended for chronic use or for the routine prophylaxis of postoperative nausea and vomiting. Domperidone should be used with great caution if given intravenously, because of the risk of arrhythmias, especially in patients predisposed to cardiac arrhythmias or hypokalaemia.

Co-administration with oral ketoconazole, erythromycin or other potent CYP3A4 inhibitors that prolong the QTc interval should be avoided (see section Interaction with other medicinal products and other form of interaction).

Pregnancy and Lactation

Domperidone Tablet should only be used during pregnancy when justified by the anticipated therapeutic benefit.

Studies have shown that domperidone enters breast milk. It is not known whether this is harmful to the newborn. Therefore, breast feeding is not recommended for mothers who are taking Domperidone Tablet.

BACK

DOMPERIDONE TABLET 10 mg

Interaction with Other Medicaments

There is a theoretical potential that domperidone may antagonize the hypoprolactinaemic effects of drugs such as bromocriptine. In addition, the prokinetic effects of domperidone may alter the absorption of some drugs. Opioid analgesics and anti-muscarinics may antagonize the prokinetic effects of domperidone.

Domperidone is metabolized via the cytochrome P450 isoenzyme CYP3A4; use with ketoconazole or erythromycin has been reported to produce increase in plasma concentrations of domperidone, and an associated slight prolongation in QT interval. Similar increases in domperidone concentrations might theoretically be seen with other potent inhibitors of CYP3A4 such as erythromycin or ritonavir, and such combinations may be best avoided.

Main Side/Adverse Effects

At the dosages and duration recommended here domperidone is generally very well tolerated with few undesirable effects.

Immune system disorder:

Very rare: allergic reactions including anaphylaxis, anaphylactic shock, anaphylactic reaction, urticaria and angioedema.

Endocrine disorder:

Rare: increased prolactin levels.

Nervous system disorders:

Very rare: extrapyramidal side effects.

Cardiac disorders:

QTc prolongation (frequency not known). Very rare (<1/10,000): ventricular arrhythmias.

Gastrointestinal disorders:

Rare: gastrointestinal disorders, including very rare transient intestinal cramps. Very rare: diarrhoea.

Skin and subcutaneous tissue disorders:

Very rare: pruritus, rash, reduced libido and other allergic reactions.

Reproductive system and breast disorders:

Rare: galactorrhoea, gynaecomastia, amenorrhoea.

Domperidone may cause an increase in prolactin levels. In rare cases, this hyperprolactinaemia may lead to neuro endocrinological side effects such as galactorrhoea, gynaecomastia and amenorrhoea. Extrapyramidal side effects are exceptional in adults. These side effects reverse spontaneously and completely as soon as treatment is stopped.

Overdose

Clinical features:

Symptoms of overdosage may include drowsiness, disorientation and extrapyramidal reactions, especially in children.

Treatment of overdosages:

There is no specific antidote to domperidone, but in the event of overdose, gastric lavage as well as the administration of activated charcoal may be useful. Close medical supervision and supportive therapy are recommended. Anticholinergic, anti parkinson drugs may be helpful in controlling the extrapyramidal reactions.

Dosage and Administration

For the treatment of nausea and vomiting:

Adults

10 to 20 mg, three or four times daily, up to a maximum daily dose of 80 mg.

Children

250 to 500 micrograms/ kg, three to four times daily, up to a maximum daily dose of 2.4 mg/kg and should not exceed a total of 80 mg daily.

For the treatment of non-ulcer dyspepsia:

Adults

10 mg, three times daily before meals and at night. The dose can be increased to 20 mg, if necessary. An initial course of treatment should not normally exceed 2 to 4 weeks.

For the treatment of migraine:

20 mg in every 4 hours (with paracetamol, if required) up to a maximum of 4 doses daily.

Storage : Store below 30°C.

Presentation/Packing : Blister pack of 1 x 14's, 7 x 14's, 10 x 10's

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